



Written Responses

Our Reference: Pre-EUA meeting, IND 19736, Amendment 125, CRMTS# 12971

DATE: November 17, 2020 **PAGES:** 8

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FROM: (b) (6)
Office of Vaccines Research and Review

SUBJECT: Type B Pre-EUA meeting to inform and obtain concurrence from the FDA regarding the proposed content (nonclinical and clinical information, and labeling) and format of the planned EUA application

PRODUCT: Human Coronavirus mRNA Vaccines [SARS-CoV-2 Spike Protein; BNT162a1 (uRNA; variant RBL063.3); BNT162b1 (modRNA; variant RBP020.3); BNT162b2 (modRNA; variant RBP020.2); and BNT162c2 (saRNA; variant RBS004.2)] in Lipid Nanoparticles (ALC-0315, ALC-0159, DSPC and Cholesterol) (COVID-19 Vaccine)

PROPOSED INDICATION: Active immunization against COVID-19 caused by SARS-CoV-2 in individuals ≥ 16 years of age

FDA Participants:

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Marion Gruber, PhD	OVRR
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We completed our review of your meeting package for COVID-19 Vaccine and are providing the following Written Responses to the questions you posed in the package.

Please be aware that your future EUA submission should include all components for a complete submission and should be in compliance with all appropriate statutes and regulations.

If you have any questions, please contact (b) (6)

Please include a reference to Pre-EUA meeting, IND 19736, Amendment 125, CRMTS# 12971 in your future submissions related to the subject product.

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Written Responses

Applicant Question 1:

Does CBER agree with the proposed content of Pfizer/BioNTech's EUA application for the candidate vaccine?

FDA Response to Question 1:

Please identify which section will contain your considerations for continuing the clinical trial following issuance of an EUA, as outlined in Section VI of our Guidance for Industry: Emergency Use Authorization for Vaccines to Prevent COVID-19. We otherwise agree with the proposed content of your EUA application for the candidate vaccine.

Applicant Question 2:

Does CBER agree with the plan to provide only the primary efficacy endpoint data in the EUA application?

FDA Response to Question 2:

We agree with the plan to provide data from the interim analysis of the primary efficacy endpoint in the EUA application. Please provide a subgroup analysis of efficacy for adolescents 16 and 17 years of age, in addition to the analyses by prespecified age stratifications.

Applicant Question 3:

Does CBER agree with the plan to provide Phase 1 and Phase 2 immunogenicity in the EUA application?

FDA Response to Question 3:

We agree with the plan to provide Phase 1 and 2 immunogenicity data in the EUA application. Please provide the data only for the secondary immunogenicity endpoints, based on the evaluable immunogenicity population, as summary tables. Please include analyses based on the all-available immunogenicity population if applicable.

Applicant Question 4:

Does CBER agree with the plan to provide all available safety data (reactogenicity, AEs, and SAEs) in the EUA application for all participants 16 years of age or older, with primary focus on the participants who have a median of 2 months of follow-up after Dose 2?

FDA Response to Question 4:

We agree with your proposal to submit of all available safety data, as outlined. Please provide a subgroup analysis of safety for adolescents 16 and 17 years of age, in addition to the analyses by prespecified age stratifications.

Applicant Question 5:

Does CBER agree with the plan to provide descriptive information on any severe COVID-19 cases accrued up to the data cutoff date for the EUA application, with continued follow-up and safety monitoring for severe cases in the ongoing Study C4591001 after EUA application filing?

FDA Response to Question 5:

We agree with your plan to provide descriptive information on any severe COVID-19 cases accrued up to the data cutoff date for the EUA application, with continued follow-up and safety monitoring for severe cases in the ongoing Study C4591001 after EUA application filing. We request that the descriptive information include a summary of cases accrued beginning at the time of randomization and first study vaccination and a summary of cases accrued during the same timeframe as for primary endpoint cases (i.e., starting 1 week after the second study vaccination). Please provide narratives of any severe COVID-19 cases that occurred in vaccine recipients from the time of randomization and first study vaccination.

Applicant Question 6:

Does CBER agree with the proposal to provide descriptive information on subpopulations of study participants (eg, age group, sex, race)?

FDA Response to Question 6:

We agree with providing descriptive information on subpopulations of study participants by age, sex and race. We will be providing a separate communication with requests for additional descriptive subgroup analyses. Please provide the analyses of safety and effectiveness by subpopulation as summary tables. Also, for analyses based on age, please define the lower limit of the age strata, e.g., 16 through 55 years of age and 56 years of age and older.

Applicant Question 7:

Does CBER agree with the proposed availability of nonclinical studies, with the QC reviewed draft report for the ongoing developmental and reproductive toxicity (DART) study to be provided as soon as possible after submitting the EUA application?

FDA Response to Question 7:

We agree with your proposal regarding the submission of reports for toxicology studies which includes a draft QC reviewed submission in mid-December for the DART study.

Applicant Question 8:

Does CBER have any comments on the proposed EUA Fact Sheets and Package Insert?

FDA Response to Question 8:

Two sets of comments regarding your Fact Sheets for Vaccination Providers and Fact Sheets for Recipients and Caregivers were sent on September 21, 2020, and

October 30, 2020. The first set of comments on your EUA Full Prescribing Information (PI) was sent on November 5, 2020. The review of your revised Fact Sheets and Full PI submitted in amendment 134 (dated November 9, 2020) is ongoing.

Applicant Question 9:

Does CBER agree with the proposed plan for active surveillance of safety for individuals who receive the Pfizer/BioNTech COVID-19 Vaccine under an EUA? And, does this fulfill the request for “active follow-up for safety” referenced in the Emergency Use Authorization for Vaccines to Prevent COVID-19 guidance (October 2020)?

FDA Response to Question 9:

Your proposed plan for active surveillance for vaccinated individuals under an EUA will be reviewed upon submission of a more detailed plan for active surveillance. At a minimum, please provide the following with your EUA application:

- a. the specific active surveillance study designs you plan to utilize, including clarification of the type of comparator group you will use to assess vaccine safety,
- b. details on how you plan to follow-up patients and identify specific outcomes, and whether these data will be collected in a prospective manner or retrospectively,
- c. whether safety outcomes will be pre-specified, or if methods will allow for the evaluation of outcomes that are not predetermined, and
- d. the minimum sample size for each study population and the study's approximate power to detect outcomes related to vaccination (which may be expected to occur at varying rates).

During review of the EUA, FDA will provide further communication on your active surveillance plans.

Applicant Question 10:

Does CBER agree with the proposed plan to continue the blinded study if an EUA is issued for the Pfizer/BioNTech COVID-19 Vaccine and to manage unblinding of any participants who elect to withdraw from the study to receive a vaccine available under an EUA?

FDA Response to Question 10:

In general, we agree with the proposed plan as outlined and recommend that you consider the option of revising the protocol to allow for a placebo crossover design to allow for subjects who wish to withdraw from the study who would be practically eligible for vaccination under the EUA, as defined in your briefing document, to receive the vaccine and continue participation in follow-up activities. This revised protocol could outline safety monitoring and additional blood draws to allow for

immunogenicity evaluations, with continued longer term follow up for safety and efficacy outcomes.

Applicant Question 11:

Pfizer/BioNTech propose to submit the following eSUB data packages (SDTM, ADaM) for studies C4591001 and BNT162-01 in the EUA:

- Complete eSUB packages (SDTM and ADaM) will be submitted based on data cut supporting the 1st successful Interim Analysis with positive efficacy results included in the EUA. It will include all Phase 2/3 subjects:
 - SDTM eSub-sub package (.xpt files, definel.xml, cSDRG, aCRF) including all SDTM datasets based on the interim analysis data cut.
 - ADaM eSub package (.xpt files, define.xml, ADRG) including relevant efficacy ADaM datasets to support the interim efficacy analyses included in the EUA, including SAS codes to support primary efficacy.
- An additional set of eSUB packages will be submitted that corresponds to the safety data cut supporting the EUA. This will include:
 - SDTM eSub package (.xpt files, define.xml, cSDRG, aCRF) including all SDTM datasets based on the safety data cut for all Phase 1/2/3 subjects.
 - ADaM eSub package (.xpt files, define.xml, ADRG) including relevant safety ADaM datasets, and related documents to support the safety analyses for all Phase 1/2/3 subjects included in the EUA.
 - SDTM and ADaM eSub packages will also be provided to support Phase 1 and 2 subjects including 1 month post dose 2 immunogenicity data.
 - An eSub package for the Phase 1 CSR can be made available upon request.
- Finally, for Study BNT162-01, we propose to submit the following eSUB package:
 - An eSUB package will be submitted based on the data cut for the abbreviated CSR. It will include all SDTM datasets and relevant ADaM datasets to support the abbreviated CSR for both BNT162b1 and BNT162b2 constructs.

Does the agency agree with this proposal?

FDA Response to Question 11:

We agree with your proposal.

Applicant Question 12:

It is Pfizer's understanding that safety and COVID-19 participant narratives and CRFs will not be required for the EUA application. Can CBER confirm?

FDA Response to Question 12:

At a minimum, we request that narratives and CRFs be provided with the EUA application for all serious adverse events and study discontinuations due to an adverse event. We may request additional information, as needed, during our review.

Applicant Question 13:

Pfizer plans to submit the safety data for all participants, including those beyond the primary scope where ongoing data collection and data cleaning activities are still in progress. Safety data cleaning for these participants will be as expected, per regular study activities following the data review plan. For transparency in data reporting, Pfizer will provide listings of all participants' adverse events, including those with outstanding queries. Does the agency agree with this approach?

FDA Response to EUA Review Process-related Question 13:

We agree with your approach. To facilitate our review, please separate the subject line listings by causality, e.g. separate lists for subjects who reported adverse events considered at least possibly related to study vaccine by the investigator or sponsor and subjects who reported adverse events that were not considered related to study vaccine by the investigator or sponsor. We may request additional information, as needed, during our review.

Applicant Question 14:

Can CBER confirm that the unblinded data in the EUA application will be considered confidential and will not be publicly or otherwise disclosed?

FDA Response to Question 14:

We plan to present to the public group-level unblinded data for safety and efficacy and may present individual-level safety information for select events of interest (e.g., suspected serious adverse reactions or severe COVID-19 cases among vaccine recipients), with attention to protecting personal health information. Please clarify any specific concerns regarding disclosure of unblinded data.

Applicant Question 15:

Can CBER provide guidance regarding data disclosure in the VRBPAC briefing materials and meeting?

FDA Response to Question 15:

Given our understanding of the anticipated timing of your final efficacy analysis, we do not have concerns regarding data disclosure in the VRBPAC briefing materials and meeting.

Applicant Question 16:

Pfizer respectfully requests to submit the fully releasable VRPBAC briefing document(s) within 5 days following the submission of the EUA application but no later than 3 days before the VRBPAC. Is this acceptable to CBER?

FDA Response to Question 16:

Our preference would be for you to submit fully releasable VRBPAC briefing document(s) with the EUA application to facilitate our review.

Applicant Question 17:

Pfizer understands that the EUA application will be a standalone document with its own application number. To facilitate development and review of the EUA document, Pfizer proposes to include in the EUA a “Cross Reference to Previously Submitted Information” that will reference the applicable information submitted to IND 19,736. Does CBER agree that Pfizer can cross reference the IND in the EUA?

FDA Response to Question 17:

Given the anticipated short time frame for our review, please only cross reference the IND for information that is complete within the IND and does not require any additional data or updates within the EUA application.

END